



Veterans Administration

VHA FAX TRANSMITTAL

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**Audie L. Murphy VA Hospital,
STVHCS**

7400 Merton Minter
San Antonio, TX 78229
210-617-5300

DATE:	Apr. 23, 2024
ATTN:	
FROM:	Community Care
SUBJECT:	Referral

Note:

Good afternoon,
Please see referral for review and scheduling

Thank you



U.S. Department
of Veterans Affairs

**VA Form 10-7080 - Approved Referral For
Medical Care**

Veteran Name: Gerold, Charles R

Veteran ICN: 1004024613V883768

Veteran EDIPI: 1342545048

Veteran Date of Birth: 1948-10-09

Veteran Preferred Name:

Pronoun:

Pronoun Description:

Veteran Address: 904 WILLIAMS ST APT 226
GONZALES, TX 78629

Veteran Phone Number: (361)407-9683

Veteran Mobile Phone Number (if Known): (830)888-0552

Veteran Business Phone Number (If Known):

Veteran Email Address (If Known):

Referral Number: VA0036698251

Priority: Routine

Referral Issue Date: 2024-03-27

Expiration Date: PRELIMINARY 2024-09-23 (SEE BELOW)*

First Appointment Date: SUPPLY TO VA ASAP

Referring VA Facility: Audie L. Murphy Memorial Veterans' Hospital

VA Telephone Number: 210-949-3850

VA Fax Number:

Initial Community Care Provider/Facility: VICTORIA DERMATOLOGY PA

Initial Provider Location: VICTORIA DERMATOLOGY PA-115 Medical Dr ; Ste 202, Victoria, TX, 77904-207N00000X

Provider Name (if known): PATEL, AMIT

Provider Phone: 281-836-4690

Provider Fax: 409-729-2449

Community Provider NPI: 1346553120

Caregiver Type:

Caregiver Details:

***IN ORDER TO PROVIDE THE FULL DURATION OF THE STANDARDIZED EPISODE OF CARE (SEOC)
PLEASE CONTACT THE REFERRING VA FACILITY WITH A FIRST APPOINTMENT TO CALCULATE
A FINAL EXPIRATION DATE. CLAIM PAYMENTS ARE DEPENDENT UPON THESE DATES.**

Any claim related to this episode of care **MUST INCLUDE THE APPROVED REFERRAL NUMBER** as the
Referral Number or Prior Authorization number.

**Please see below for Additional VA Referring Facility Information, Billing Information
and Appointment/Provider Information.**

Pertinent Clinical Information

**Please view the Clinical Information in the VA Order section for more information related to the Original VA Order
Reason for Request.**

Chief Complaint: -Pt and daughter state about a week and a half ago they noticed some redness and crust to the
borderlines of the area where he had the melanoma.

Patient History / Clinical Findings / Diagnosis (Co-Morbidities): n/a Is this referral for Service Connected condition?
No Per VA PBM Formulary Process Management Policy from VHA Handbook 1108.08, the use of drugs solely to improve

normal physiologic function or to enhance body appearance is generally not considered medically necessary and therefore are not to be prescribed. Medications used for this purpose are considered cosmetic. The use of medications for hair re-growth (e.g., minoxidil, finasteride, or pimecrolimus) are considered cosmetic in nature and should not be prescribed unless care is being provided in connection with the treatment of a service- connected injury.

Provisional Diagnosis: C434 Malignant melanoma of scalp and neck

Services Authorized

The VA Order Reason for Request is the official clinical order. This scope of services associated with the medical care for this authorization is found below. Necessary services that are not included must be requested using the Request for Services procedures. Please visit the VHA Storefront www.va.gov/COMMUNITYCARE/providers/index.asp for additional resources and requirements.

Service Requested: Dermatology_PRCT SEOC 1.0.12

Category of Care: DERMATOLOGY

Procedural Overview - Standardized Episode of Care (SEOC)

Dermatology_PRCT SEOC 1.0.12 Duration: 180 Days

Description:

This authorization covers services associated with the specialty(s) identified for this episode of care, including all medical care listed below relevant to the referred care specified on the consult/referral order.

NOTE: Please note this SEOC does not cover cosmetic procedures that are not considered medically necessary or correct a functional disability as they are excluded from the Medical Benefits Package.

NOTE: This SEOC does not cover Mohs. A separate referral specifically for Mohs with the Mohs SEOC is required.

NOTE: Superficial Radiation Therapy (SRT) is not covered in this episode of care. SRT is only to be performed and approved by a radiation oncologist and not a dermatologist. SRT must be requested through an RFS and approved using one of the radiation therapy SEOCs.

NOTE: Additional consultations needed relevant to the patient complaint/condition require VA review and approval including a referral to another specialty for secondary/complex closure.

NOTE: This SEOC does not cover the removal of skin tags, seborrheic keratosis, and other benign skin lesions unless they are bleeding, painful, or changing.

No.	Service/Procedure	Number Of Visits Authorized
1	Initial outpatient evaluation (including full body exam), treatment, and follow-up visits for the referred condition on the consult/referral order	999
2	Diagnostic imaging relevant to the referred condition on the consult/referral order	999
3	Labs and pathology relevant to the referred condition on the consult/referral order	999
4	Procedures performed by the dermatology provider relevant to the referred condition on the consult/referral order including but not limited to: cryotherapy, biopsy (e.g., excisional,	999

shave, punch), excision, repair, excimer laser, photodynamic therapy, UVB therapy, debridement, medically necessary hair removal

Additional Information:

- * Please visit the VHA Storefront www.va.gov/COMMUNITYCARE/providers/index.asp for additional resources and requirements pertaining to the following:
- * Pharmacy prescribing requirements
- * Durable Medical Equipment (DME), Prosthetics, and Orthotics prescribing requirements
- * Precertification (PRCT) process requirements
- * Request for Services (RFS) requirements

REFER ALL QUESTIONS RELATED TO THIS APPROVAL TO THE ISSUING VA OFFICE

Referring VA Facility: Audie L. Murphy Memorial Veterans' Hospital

Station Number: 671

Telephone Number: 210-949-3850

Address: 7400 Merton Minter Boulevard SAN ANTONIO TX 78229

Referring Provider: HOHLT, RUSSELL

Referring Provider NPI: 1497988109

Unique Consult No: 671_8526453

Program Authority: Authorized/Pre-authorized VA Referral (not otherwise specified) - 1703

Affiliation: Triwest

Network: CC Network 4

Appointments/Providers Assigned to the Referral

Provider/Facility Name	Provider/Facility Location	Appt Date	Appt Time	Telephone #	Fax #
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Additional Service Information**Request for Services (RFS)**

A Request for Services (RFS) is a provider-generated request for new or additional care outside the scope of the current approved referral/authorization. Provider should always submit the RFS directly to the authorizing VAMC, preferably via the HSRM portal. Providers should always submit an RFS on the same day it is determined it's needed and before delivering care, unless it is emergent care. In that case, the RFS can be submitted simultaneously.

How to Submit an RFS

VA prefers providers submit an RFS via the HSRM portal, available on the VA's website

- Go to the VA Storefront at <https://www.va.gov/COMMUNITYCARE/providers/Care-Coordination.asp#RFS>
 - Navigate to the link to the RFS form at the bottom of the section

Medical Records and Documents Requirements

Medical Records and documentation are required for all provider services. Providers are required to submit Medical documentation directly to the authorizing VAMC, preferably via upload to HSRM.

Billing and Other Referral Information**Submitting Claims**

ANY CLAIMS RELATED TO THIS EPISODE OF CARE MUST BE SUBMITTED TO TRIWEST OR DELTA DENTAL AS APPROPRIATE AND INCLUDE THE APPROVED REFERRAL NUMBER.

Methods to submit claims:**Electronic Data Interchange (EDI):**

Payer ID for Medical – TWVACCN

Payer ID for Dental – CDCA1

More information on how to submit claims can be found by visiting

<https://www.va.gov/COMMUNITYCARE/revenue-ops/Veteran-Care-Claims.asp>

Precertification

The Standardized Episode of Care (SEOC) referral you have accepted does not include services that require third-party payer (TPP) precertification.

Community Care Medical Policies

VA Community Care Medical Policies describe standard VA health care benefits for services and procedures that community providers may recommend as necessary for a Veteran. Prior to providing care, providers should use the Community Care Medical Policies as a reference when determining if a Veteran meets VA clinical criteria. When additional services are requested, Community Care Medical Policies will be used to determine approval by a clinical reviewer. Community Care Medical Policies and supporting information can be found at:

<https://www.va.gov/COMMUNITYCARE/providers/info-CDI.asp>

Medical Devices

As part of furnishing the services authorized on the approved referral, you may recommend that medical devices, adaptive equipment, or other items be provided for the treatment or rehabilitation of the Veteran's medical condition. In such instances, you must complete a Request for Services (RFS) as outlined above.

<https://www.va.gov/COMMUNITYCARE/providers/Care-Coordination.asp#RFS>

The RFS must (1) indicate that the condition for which the item is being prescribed is within the scope of the authorized services on the approved referral, (2) describe the item or service being prescribed with as much specificity as possible (including manufacturer and model, needed custom measurements, size), and (3) provide a brief but thorough plain-language explanation of how the item or service will serve the treatment or rehabilitation needs of the Veteran. All parts of the RFS form must be filled out completely.

VA staff will determine whether the prescribed item or service is one that VA is authorized to purchase. If the item is not something that we are authorized to purchase, VA staff will work with you to identify an alternative that will best meet the Veteran's clinical needs. In making this determination, VA staff will consider your clinical judgment and expertise and work with you to resolve outstanding issues quickly.

In the event of an urgent or emergent need, you may provide medical devices, necessary items or services prescribed to Veterans receiving care in the community for urgent or emergent conditions at the time of healthcare service delivery or soon thereafter. Urgent or emergent DME or Medical Devices may include, but are not limited to splints, crutches, canes, slings, soft collars, walkers, and manual wheelchairs. DME dispensed directly to the Veteran based on an urgent or emergent need will be paid by the CCN Third Party Administrator. Claims for these items should be submitted using the process outlined in the **Submitting Claims** section of this form.

Pharmacy**Drug Safety and Administration Requirements**

• Must follow the VA National Protocol and clinical guidance for Esketamine or Ketamine administration. Ketamine treatments for mental health or for pain are not approved under a CCN referral. Prior to administration it is required the ordering and administering providers review the VA Protocol and Clinical Guidance found through the VA Formulary Search Tool available here:

• <https://www.pbm.va.gov/apps/VANationalFormulary/>

Must follow the VA Opioid Safety Guidelines and complete the required Opioid Safety Training found here:

• <https://www.pbm.va.gov/apps/VANationalFormulary/>

• <https://www.va.gov/COMMUNITYCARE/providers/EDU-Training.asp>

CVS Caremark is the retail pharmacy network for Veterans' immediately needed or Urgent/Emergent prescriptions.

Immediate need prescriptions:

- Must follow the VA Urgent/Emergent Formulary which can be found at <http://www.pbm.va.gov/PBM/nationalformulary.asp>
- Prescription can only go up to a 14-day supply. No refills of the immediate need medication may be authorized.
- Only a seven-day supply for opioids, or up to the opioid prescribing limit allowed by State—whichever is less—may be authorized.

Immediate need prescription extending past 14 days:

- The provider will need to send second prescription (beyond 14 days) to the referring VA medical facility's pharmacy for prescription fulfillment services.

Routine/maintenance prescriptions:

- Must be sent to the referring VA medical facility's pharmacy

If you do not have the ability to electronically submit prescriptions to pharmacies, please contact the Community Care representative at the referring VA medical facility for their pharmacy fax number. Please refer to: <https://www.va.gov/COMMUNITYCARE/providers/Service-Requirements.asp> for additional instructions related to prescriptions.

Clinical Information on the VA Order

Reason for Request:

Provider to receive results:hohlt

Type of Service: Evaluation and Treatment

Chief Complaint:

-Pt and daughter state about a week and a half ago they noticed some redness and crust to the borderlines of the area where he had the melanoma.

Patient History / Clinical Findings / Diagnosis (Co-Morbidities):

n/a

Is this referral for Service Connected condition? No

Per VA PBM Formulary Process Management Policy from VHA Handbook 1108.08, the

use of drugs solely to improve normal physiologic function or to enhance body

appearance is generally not considered medically necessary and therefore are not

to be prescribed. Medications used for this purpose are considered cosmetic.

The use of medications for hair re-growth (e.g., minoxidil, finasteride, or

pimecrolimus) are considered cosmetic in nature and should not be prescribed

unless care is being provided in connection with the treatment of a service-

connected injury.

****This 10-7080 - Approved Referral For Medical Care was generated on 04/23/2024 changes made to the referral after**

this date are not reflected on the form.



U.S. Department of Veterans Affairs

Veterans Health Administration
Office of Community Care

Health Share Referral Manager (HSRM): VA's Secure Online Portal for Managing Referrals and Authorizations

Why are community providers across the country excited to use HSRM?

- ◆ **Facilitates Health Information Exchange (HIE)** between community providers and VA via a unified platform
- ◆ Conveniently **organizes all active referrals** in one centralized location
- ◆ **Reduces time spent** waiting for fax, phone or email contact
- ◆ Allows community providers to **request authorization** for additional services or additional time to provide services
- ◆ Promotes **reduced turnaround time** for authorizations and reimbursement

HSRM Account Creation For Community Providers

- ◆ **STEP 1: Training:** Each staff member attends virtual training, completes eLearning lessons, or reviews one of the training guides
- ◆ **STEP 2: ID.me:** Each staff member creates an ID.me account and verifies their identity at <https://www.id.me/>
- ◆ **STEP 3: Submit EUT:** One facility point of contact (POC) fills out the End User Tracker (EUT), then sends it to HSRMSupport@va.gov
- ◆ **STEP 4: Receive Accounts:** Help Desk creates accounts in HSRM, then provides confirmation of account creation to the facility POC
- ◆ **STEP 5: Log Into HSRM:** Each staff member logs into HSRM at <https://ccracommunity.va.gov>
- ◆ Once these steps are complete, contact the VA Medical Center(s) you work with to let them know you have access to HSRM and to discuss your transition to HSRM

HSRM Registration Resources

- ◆ HSRM Support Points of Contact: <https://www.va.gov/COMMUNITYCARE/providers/HSRM-POC-List.asp>
- ◆ ID.me Registration Instructions: https://www.va.gov/COMMUNITYCARE/docs/pubfiles/factsheets/FactSheet_26-06.pdf#
- ◆ End User Tracker
 - End User Tracker Template: https://www.va.gov/COMMUNITYCARE/docs/providers/HSRM_End_User_Tracker.xlsx#
 - Send completed End User Tracker to HSRMSupport@va.gov
- ◆ HSRM Login: <https://ccracommunity.va.gov>

HSRM Educational Resources

- ◆ Live Virtual Training: https://www.train.org/virginia/course/1082953/live_event; please visit link to find available dates and to register in advance
- ◆ HSRM eLearning lessons: Log into <https://www.train.org/vha/welcome>, then search the course catalogue for "HealthShare Referral Manager"
- ◆ HSRM User Guide: https://www.va.gov/COMMUNITYCARE/docs/providers/HSRM_User_Guide.pdf
- ◆ HSRM Quick Reference Guide (QRG): https://www.va.gov/COMMUNITYCARE/docs/providers/HSRM_Quick_Reference_Guide.pdf#
- ◆ Additional information about HSRM: https://www.va.gov/COMMUNITYCARE/providers/Care_Coordination.asp
- ◆ HSRM Help Desk
Phone: 1-844-293-2272, Email: HSRMSupport@va.gov, Hours: Monday - Friday 7:30 a.m. - 5 p.m. Eastern Time



Veterans Health Administration

VHA Medical Documentation

Document Created: 04/23/2024 16:23

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AUDIE L MURPHY MEMORIAL VAMC 7400 Merton Minter Blvd, San Antonio, Texas 78229-4404
VA Referrals phone: 210-949-3850 Fax:

Point of Contact (POC): MARTINEZ, GRETCHEN
POC Phone: 2109493850 Fax:

Patient Name: GEROLD,CHARLES R
904 WILLIAMS ST APT 226
GONZALES, TEXAS 78629

DOB: 10/09/1948
Phone (residential): (361)407-9683
Phone (mobile): (830)888-0552

Referral Type: Community Care
Network

Next of Kin contact information

PINEDA,ROSIE

Address (Next Of Kin)
1612 FLINT
WACO, TEXAS 76706

Phone: (830)888-0552
Phone (work): (830)463-2778

CONSULTS

Current PC Provider: HOHLT,RUSSELL W
Current PC Team: VICTORIA BLUE TEAM *WH*
Current Pat. Status: Outpatient
Primary Eligibility: NSC, VA PENSION
UCID: 671_8526453
Patient Type:
OEF/OIF: NO

Order Information

To Service: COMMUNITY CARE-DERMATOLOGY
Attention:
From Service: VOP CHART CHECK
Requesting Provider: HOHLT,RUSSELL W
Service is to be rendered on an OUTPATIENT basis
Place: Consultant's choice
Urgency: Routine
Clinically Ind. Date: 2024-04-16 00:00:00

DST ID:
 Orderable Item:
 Consult: Consult
 Provisional Diagnosis: Malignant Melanoma Of Scalp And Neck (ICD-10-CM C43.4)
 Reason For Request:
 Provider to receive results:hohlt
 Type of Service: Evaluation and Treatment

Chief Complaint: -Pt and daughter state about a week and a half ago they noticed some redness and crust to the borderlines of the area where he had the melanoma.

Patient History / Clinical Findings / Diagnosis (Co-Morbidities):

n/a

Is this referral for Service Connected condition? No

Per VA PBM Formulary Process Management Policy from VHA Handbook 1108.08, the use of drugs solely to improve normal physiologic function or to enhance body appearance is generally not considered medically necessary and therefore are not to be prescribed. Medications used for this purpose are considered cosmetic. The use of medications for hair re-growth (e.g., minoxidil, finasteride, or pimecrolimus) are considered cosmetic in nature and should not be prescribed unless care is being provided in connection with the treatment of a service- connected injury.

Inter-facility Information

AUDIE L. MURPHY MEMORIAL HOSP

Status: ACTIVE

Last Action: RECEIVED

Facility

Activity	Date/Time/Zone	Responsible Person	Entered By

CPRS RELEASED ORDER	03/26/24 15:05	HOHLT,RUSSELL W	HOHLT,RUSSELL W
ADDED COMMENT	03/26/24 00:00	HOHLT,RUSSELL W	HOHLT,RUSSELL W
RECEIVED	03/27/24 13:50	LARS,SUSAN M	LARS,SUSAN M

Note: TIME ZONE is local if not indicated

No local TIU results or Medicine results available for this consult

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===== END =====

PROBLEM LIST

Sensitive Diagnoses

No sensitive diagnoses were provided.

Other Diagnoses

Problem	Code
Carcinoma in situ of skin, unspecified	D04.9
Cerebellar stroke syndrome	G46.4
Cerebral infarction, unspecified	I63.9
Chronic obstructive pulmonary disease, unspecified	J44.9
Essential (primary) hypertension	I10.
Hyperlipidemia, unspecified	E78.5

MEDICATIONS

100 most recent outpatient medications released by VA to Veteran in the last 6 months

Medication Name and Dose	Quantity	Refill Number	Issue and Fill Date	Status
CLOPIDOGREL BISULFATE 75MG TAB TAKE ONE TABLET BY MOUTH EVERY DAY FOR BLOOD CLOT PREVENTION	Qty:90	Fill: 1 of 3	Orig: 2023-11-20 Last: 2024-02-08	ACTIVE
GABAPENTIN 300MG CAP TAKE TWO CAPSULES BY MOUTH THREE TIMES A DAY FOR NERVE PAIN	Qty:360	Fill: 3 of 3	Orig: 2023-08-11 Last: 2024-02-22	ACTIVE
LISINAPRIL 40MG TAB TAKE ONE TABLET BY MOUTH EVERY DAY FOR BLOOD PRESSURE	Qty:90	Fill: 1 of 3	Orig: 2024-02-22 Last: 2024-03-04	ACTIVE
METOPROLOL TARTRATE 100MG TAB TAKE ONE-HALF TABLET BY MOUTH TWICE A DAY FOR BLOOD PRESSURE/HEART	Qty:90	Fill: 3 of 3	Orig: 2023-06-20 Last: 2024-03-06	ACTIVE
MULTIVITS W/MINERALS TAB/CAP (NO VIT K) TAKE 1 TABLET BY MOUTH EVERY DAY	Qty:100	Fill: 1 of 2	Orig: 2023-09-28 Last: 2024-04-03	ACTIVE
SIMVASTATIN 80MG TAB TAKE ONE-HALF TABLET BY MOUTH AT BEDTIME FOR CHOLESTEROL	Qty:45	Fill: 1 of 3	Orig: 2023-11-20 Last: 2024-04-03	ACTIVE
TRAZODONE HCL 50MG TAB TAKE ONE TABLET BY MOUTH AT BEDTIME FOR SLEEP	Qty:30	Fill: 3 of 8	Orig: 2024-01-12 Last: 2024-04-08	ACTIVE
TRAZODONE HCL TAB 50 MG TAKE ONE TABLET BY MOUTH AT BEDTIME FOR SLEEP	Qty:30	Fill: 8 of 8	Orig: 2023-02-21 Last: 2023-12-11	DISCONTINUED
ASPIRIN 81MG CHEW TAB CHEW ONE TABLET BY MOUTH EVERY DAY FOR HEART/ TO THIN BLOOD	Qty:90	Fill: 3 of 3	Orig: 2023-02-07 Last: 2023-11-13	EXPIRED

ALLERGIES

Name	Origin	Verified
BACLOFEN	Origin: 2013-02-25	Verified: 2013-02-25

PROGRESS NOTES

LOCAL TITLE: VOPC PHYSICIAN NOTE
STANDARD TITLE: PHYSICIAN NOTE
DATE OF NOTE: DEC 22, 2023@12:07 ENTRY DATE: DEC 22, 2023@12:07:29
AUTHOR: HOHLT,RUSSELL W EXP COSIGNER:
URGENCY: STATUS: COMPLETED

DATE: DEC 22, 2023

GEROLD,CHARLES R, ***_**_****
Age: 75 Sex: MALE Race: WHITE, NOT OF HISPANIC ORIGIN

Weight: 165 lb [74.84 kg] (12/22/2023 11:48) Height: 66 in [167.6 cm]
(12/22/2023 11:48)
Respiration: 20 (12/22/2023 11:48) Temperature: 97.7 F [36.5 C] (12/22/2023
11:48)
Pulse: 66 (12/22/2023 11:48) Blood Pressure: 136/74 (12/22/2023 11:50)

Pain Scale: 0 (12/22/2023 11:48)

DISCLAIMER: For patients being seen at DOD, Cerner Sites, and VHA facilities, Remote Medications, Remote allergies and adverse drug reactions (ADRs) may be incomplete. Patient medication list and allergies must be viewed using Joint Legacy Viewer (JLV). For patients seen at DOD, Cerner Sites and VHA facilities,

I have reviewed patient medications using JLV.

ALLERGIES REMOTE AND LOCAL REVIEW:
FACILITY ALLERGY/ADR

No Remote Allergy/ADR Data available for this patient
AUDIE L. MURPHY MEMORIAL HOSP BACLOFEN

Remote and Local Allergies reviewed, and reactions confirmed.

Medications:

Active Inpatient, Outpatient and Clinic Medications (including Supplies):

Active Outpatient Medications	Status	Issue Date	Last Fill	Refills	Expiration
1) ACETAMINOPHEN 500MG TAB Qty: 100 for 30 days Sig: TAKE ONE TABLET BY MOUTH EVERY 8 HOURS AS NEEDED FOR PAIN **DO NOT EXCEED 4000MG ACETAMINOPHEN DAILY*	ACTIVE	Issu:08-25-23	Last:08-28-23	Refills: 2	Expr:08-25-24
2) ASPIRIN 81MG CHEW TAB Qty: 90 for 90 days Sig: CHEW ONE TABLET BY MOUTH EVERY DAY FOR HEART/TO THIN BLOOD	ACTIVE	Issu:02-07-23	Last:11-13-23	Refills: 0	Expr:02-08-24
3) CLOPIDOGREL BISULFATE 75MG TAB Qty: 90 for 90 days Sig: TAKE ONE TABLET BY MOUTH EVERY DAY FOR BLOOD CLOT PREVENTION	ACTIVE	Issu:11-20-23	Last:11-20-23	Refills: 3	Expr:11-20-24
4) GABAPENTIN CAP 300MG Qty: 360 for 60 days Sig: TAKE TWO CAPSULES BY MOUTH THREE TIMES A DAY FOR NERVE PAIN	ACTIVE	Issu:08-11-23	Last:12-02-23	Refills: 1	Expr:08-11-24
5) LISINAPRIL 40MG TAB Qty: 90 for 90 days Sig: TAKE ONE TABLET BY MOUTH EVERY DAY FOR BLOOD PRESSURE	ACTIVE	Issu:03-20-23	Last:12-05-23	Refills: 0	Expr:03-20-24
6) METOPROLOL TARTRATE 100MG TAB Qty: 90 for 90 days Sig: TAKE ONE-HALF TABLET BY MOUTH TWICE A DAY FOR BLOOD PRESSURE/HEART	ACTIVE	Issu:06-20-23	Last:12-07-23	Refills: 1	Expr:06-20-24
7) MULTIVITS W/MINERALS TAB/CAP (NO VIT K) Qty: 100 for 90 days Sig: TAKE 1 TABLET BY MOUTH EVERY DAY	ACTIVE	Issu:09-28-23	Last:12-15-23	Refills: 2	Expr:09-28-24
8) SIMVASTATIN 80MG TAB Qty: 45 for 90 days Sig: TAKE ONE-HALF TABLET BY MOUTH AT BEDTIME FOR CHOLESTEROL	ACTIVE	Issu:11-20-23	Last:12-20-23	Refills: 3	Expr:11-20-24
9) TRAZODONE HCL TAB 50 MG Qty: 30 for 30 days Sig: TAKE ONE TABLET BY MOUTH AT BEDTIME FOR SLEEP	ACTIVE	Issu:02-21-23	Last:12-11-23	Refills: 0	Expr:02-22-24

Medication Reconciliation:

I have performed med rec with the patient/caregiver utilizing the Essential Medication List for Review (EMLR) MRT1. This included, under MRR1 active and

pending prescriptions dispensed from local and remote VA prescriptions, DoD prescriptions, local non-VA medications, local inpatient medications orders, clinic medication orders and prescriptions that have expired or been discontinued in the past 90 days. There were no relevant meds aside from those listed above.

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MEDICATION REVIEW

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No changes made to medications during this visit.

Problem List:

Active problems - Computerized Problem List is the source for the following:

1. Cancer in situ of skin
2. HLD - Hyperlipidemia
3. COPD - Chronic Obstructive Pulmonary Disease (SCT 13645005)
4. Hypertension
5. Cerebral infarction
6. Unsteady when walking

LABS:

CBC:

Collection DT	Specimen	Test Name	Result	Units	Ref
Range					
06/13/2023 08:26	BLOOD	WBC	9.7	10.e3/uL	4.0 - 10.0
06/13/2023 08:26	BLOOD	RBC	4.20 L	10.e6/uL	4.5 - 5.9
06/13/2023 08:26	BLOOD	HGB	13.3 L	g/dL	13.5 - 17.5
06/13/2023 08:26	BLOOD	HCT	40.7 L	%	41.0 - 53.0
06/13/2023 08:26	BLOOD	MCV	96.9	fL	78.0 - 98.0
06/13/2023 08:26	BLOOD	MCH	31.6	pg	26.0 - 34.0
06/13/2023 08:26	BLOOD	MCHC	32.7	gm/dL	32.0 - 36.0
06/13/2023 08:26	BLOOD	PLTS	311	10.e3/uL	150 - 400

Chem 20:

Date	Test	Result	Units	Normal Range	Specimen
06/13/2023	CREATININE	0.8	mg/dL	.7 - 1.3	PLASMA
06/13/2023	UREA NITROGEN	10	mg/dL	7 - 25	PLASMA
06/13/2023	GLUCOSE	82	mg/dL	70 - 105	PLASMA
06/13/2023	SODIUM	134 L	mmol/L	136 - 145	PLASMA
06/13/2023	POTASSIUM	4.4	mmol/L	3.5 - 5.1	PLASMA
06/13/2023	CHLORIDE	99	mmol/L	98 - 107	PLASMA
06/13/2023	CALCIUM	8.8	mg/dL	8.6 - 10.3	PLASMA
06/13/2023	PROTEIN,TOTAL	7.4	g/dL	6.4 - 8.9	SERUM
06/13/2023	ALBUMIN	3.9		3.5 - 5.7	SERUM
06/13/2023	TOT. BILIRUBIN	0.5		0.3 - 1.0	SERUM
06/13/2023	ALKALINE PHOSP	80	U/L	34 - 104	SERUM
06/13/2023	AST	22	IU/L	13 - 39	SERUM
06/13/2023	ALT	27	IU/L	7 - 52	SERUM
06/13/2023	ANION GAP	9.0	mmol/L		PLASMA

06/13/2023	CO2	26.0	mmol/L	21 - 31	PLASMA
06/13/2023	ICTERUS	0		NEG	PLASMA
06/13/2023	LIPEMIA	0		NEG	PLASMA
06/13/2023	HEMOLYSIS	0		NEG	PLASMA
06/13/2023	OSMOLALITY-CAL	266.40	mOsm/kg		PLASMA
11/02/2021	zzEGFR	>60			PLASMA
06/13/2023	EGFR CKD EPI	93			PLASMA

A1C:

Date	Test	Result	Units	Normal Range	Specimen
06/13/2023	HEMOGLOBIN A1C	6.3 H	%	4.0 - 6.0	BLOOD

PROSTATIC SPECIFIC ANTIGEN 7/21/23 13:34 3.64

LIPID PROFILE Coll. dat6/13/23 08:26 12/21/22 10:01 5/2/22 11:37

CHOL	117	121	123
TRIG	104	97	116
HDL	50	52	51
LDL CHOLESTEROL	46.00 L	50.00 L	49.00 L
VLDL CHOL	21.0	19.0	23.0
LDL DIRECT			

Chief complaint

- see nursing not
- would like flu vacc

This is a 6-month follow-up appointment.

Subjective

This patient has history of CVA with unsteady gait from his left lower extremity, essential hypertension, dyslipidemia, skin cancer who is seen Dr. Guoghi for care. He is taking all of his medications without any side effects.

He has no complaints today.

His blood pressure and pulse are normal. Pulse: 66 (12/22/2023 11:48) Blood Pressure: 136/74 (12/22/2023 11:50)

I did answer all of his questions.

Review of systems

No chest pain, dyspnea, abdominal pain, nausea, vomiting, fever, chills, dizziness, lightheaded, sore throat, cough, diarrhea, constipation

Physical exam

General the patient is alert and oriented x4

Psych no suicidal or homicidal thoughts

Neck is supple no cervical lymphadenopathy

Lungs clear to auscultation bilaterally no rhonchi or wheezes

Heart normal rate normal rhythm S1-2 heard

Abdomen normal bowel sounds at all 4 quadrants, no rebound, tenderness, guarding

Extremities show no cyanosis no clubbing and no edema

Skin warm and dry

Left upper extremity by his left side due to stroke with weakness

He will do his labs today.

Assessment and plan

History of CVA with left-sided weakness arms and legs in a wheelchair with hypertension. He does weigh approximately same as 6 months ago.

Essential hypertension continue his hypertensive medications metoprolol 50 mg twice daily and lisinopril 40 mg daily, aspirin 81 mg daily, Plavix 75 mg daily and statin simvastatin 40 mg nightly. Stable

Unsteady gait with CVA with left-sided weakness arms and legs he does use a wheelchair. Continue gabapentin 300 mg 3 times a day. Stable

Sleep disorder continue trazodone 50 mg nightly. Stable

Disposition:

I have reviewed recent test results at length with patient. I did give him the opportunity to assess questions, and answered all of his questions with satisfaction. Patient verbalized understanding of all information and instructions.

I have instructed patient to bring all of his medication bottles at the next appointment for complete medication reconciliation. Patient should include the OTC medications as well as prescribed medication.

100% of this time was spent evaluating this patient, as well as counseling and coordinating the care addressing conditions, and she about the course of the disease, answering questions, making recommendations, prescribing medications with appropriate and entered the necessary referrals to optimize care, as necessary.

DC home

continue medications, as prescribed.

RETURN TO CLINIC

- 6 months with PCP and repeat labs CBC, Chem-12, hemoglobin A1c, TSH, lipid profile, urinalysis

Dragon Speak Clarification: Dragon Speak dictation program was used by this author for this note. Attempts at proofreading are not always successful at identifying errors and word transposition may occur. Please contact writer if significant errors are present.

He is not with the testing

CLINICAL REMINDER ACTIVITY

Hepatitis C Testing:

Patient declines HCV lab test.

Reason: n/a

/es/ RUSSELL W HOHLT
MD, Chief Medical Officer
Signed: 12/22/2023 12:26

Department of Veterans Affairs		REQUEST FOR SERVICES (RFS) FORM	
PREVIOUS AUTHORIZATION NUMBER: TODAY'S DATE (MM/DD/YYYY):		NOTE: The Request for Services (RFS) Form 10-10172 must be submitted via an approved method (HSRM, Electronic fax, Direct Messaging, traditional fax, or mail). Completion of this form is REQUIRED and MUST BE SIGNED by the requesting provider for further care to be rendered to a Veteran patient.	
SECTION I: VETERAN INFORMATION			
1. VETERAN'S LEGAL FULL NAME (First, MI, Last):		2. DOB (MM/DD/YYYY):	
3. VA FACILITY: STVHCS/Pink Team - Team Fax: (210) 443-0301		4. VA LOCATION: San Antonio, TX Ph: (210) 949-3850	
SECTION II: ORDERING PROVIDER INFORMATION			
5. REQUESTING PROVIDER'S NAME:		6. NPI #:	7. SPECIALTY:
8. OFFICE NAME & ADDRESS:			
9. SECURE EMAIL ADDRESS:			
10. PHONE NUMBER:	11. FAX NUMBER:		☐ 12. INDIAN HEALTH SERVICES (IHS) PROVIDER?
SECTION III: TYPE OF CARE REQUEST			
13. PLEASE INDICATE CLINICAL URGENCY (Urgent care is only applicable for requests that require less than 3 days to process. If care is needed within 48 hours or if Veteran is at risk for Suicide/Homicide, please call the VA directly on the same day as completed RFS form submission. Do NOT mark urgent for administrative urgency): ☐ ROUTINE ☐ URGENT			
14. DIAGNOSIS (ICD-10 Code/Description):		15. DATE OF SERVICE (MM/DD/YYYY) &/OR ANTICIPATED LENGTH OF CARE:	
16. CPT/HCPCS CODE &/OR DESCRIPTION OF REQUESTED SERVICES (Include units/visits, add second list page, if needed):			
17. HOW MANY VISITS HAVE OCCURRED SO FAR? (If known)		18. IS THIS A REFERRAL TO ANOTHER SPECIALTY? ☐ YES (If "YES," please fill out the Servicing Provider/Specialty information below) ☐ NO	
19. SERVICING PROVIDER'S NAME:		20. NPI #:	21. SPECIALTY:
22. OFFICE NAME & ADDRESS:			
23. SECURE EMAIL ADDRESS:			
24. PHONE NUMBER:		25. FAX NUMBER:	
SECTION IV: TYPE OF SERVICE REQUESTED			
26. OUTPATIENT CARE: ☐ PT ☐ OT ☐ SPEECH THERAPY		27. SURGICAL PROCEDURE: ☐ INPATIENT ☐ OUTPATIENT	
FREQUENCY & DURATION:		FACILITY NAME:	
28. ☐ IN-OFFICE PROCEDURE		29. INPATIENT CARE: ☐ LTACH ☐ ACUTE REHAB ☐ BH	
30. ☐ ADDITIONAL OFFICE VISITS (List # needed):		31. ☐ EXTENSION OF VALIDITY DATES	
32. ☐ EMERGENCY ROOM CARE		33. ☐ LABS (If done outside of office, please provide facility above)	
34. ☐ RADIOLOGY/IMAGING (If done outside of office, please provide facility above)		35. ☐ PRE-OPS LABS ☐ CHEST XRAY ☐ EKG ☐ OTHER:	
36. JUSTIFICATION FOR REQUEST (To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).			

VETERAN'S LEGAL FULL NAME (First, MI, Last):		
SECTION V: GERIATRICS AND EXTENDED CARE SERVICES (If applicable)		
37. ☐ COMMUNITY ADULT DAY HEALTH CARE ☐ COMMUNITY NURSING HOME ☐ HOMEMAKER/HOME HEALTH AIDE ☐ HOME INFUSION ☐ HOSPICE/PALLIATIVE CARE ☐ RESPITE ☐ SKILLED HOME HEALTH CARE ☐ OTHER: _____		
FREQUENCY & DURATION: _____		
38. JUSTIFICATION FOR REQUEST (To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).		
SECTION VI: HOME OXYGEN INFORMATION (If applicable)		
39. PAO2 AT REST:	40. O2 SAT AT REST:	41. OXYGEN FLOW RATE:
42. EXTENT OF SUPPORT (Continuous, Intermittent, Specific Activity):		
43. OXYGEN EQUIPMENT (Stationary/Portable):		
44. DELIVERY SYSTEM (Cannula, Mask, Other):		
SECTION VII: DME & PROSTHETICS INFORMATION (If applicable)		
45. HCPS FOR THE ITEM(S) BEING PRESCRIBED:		
46. BRAND, MAKE, MODEL, PART NUMBERS:		
47. MEASUREMENTS:		
48. QUANTITY:	49. ICD-10:	50. PROVISIONAL DIAGNOSIS:
51. DELIVERY/PICKUP OPTIONS: ☐ DELIVER TO ORDERING PROVIDER'S ADDRESS ☐ VETERAN WILL PICKUP AT THE VA MEDICAL CENTER ☐ DELIVER TO COMMUNITY VENDOR FOR DELIVERY & SETUP FOR DME ☐ DELIVER TO VETERAN'S HOME		
SECTION VIII: DURABLE MEDICAL EQUIPMENT (DME) EDUCATION & TRAINING (If applicable)		
Please see DME Requirements/Pharmacy Requirements - Community Care (va.gov) for URGENT DME requests.		
NOTE: Failure to thoroughly complete the RFS for DME will result in delayed patient care & prevent the VA from DME fulfillment.		
52. BEFORE DME WILL BE ISSUED, EDUCATION, TRAINING, &/OR FITTING OF DME (as applicable for the specific DME being ordered) TO THE VETERAN MUST BE COMPLETE. PLEASE INDICATE WHETHER THE FOLLOWING HAS BEEN COMPLETED FOR THE VETERAN. NOTE: If not completed, DME will be mailed to requesting provider's address to coordinate an alternative time for proper instruction on DME use.		A. EDUCATION: ☐ YES ☐ NO B. TRAINING: ☐ YES ☐ NO ☐ N/A C. FITTING: ☐ YES ☐ NO ☐ N/A
53. JUSTIFICATION FOR REQUEST (To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).		

VETERAN'S LEGAL FULL NAME (First, MI, Last):	
SECTION IX: THERAPEUTIC FOOTWEAR ASSESSMENT INFORMATION (If applicable)	
54. FILL OUT THE INFORMATION BELOW (If applicable): ☐ LEFT FOOT ☐ RIGHT FOOT ☐ BILATERAL ☐ PREFABRICATED THERAPEUTIC FOOTWEAR ☐ CUSTOM THERAPEUTIC FOOTWEAR NOTE: For prescription of therapeutic footwear for severe or gross foot deformity which cannot be accommodated with conventional footwear. DESCRIBE FOOT DEFORMITY AND ADDITIONAL DETAILS:	NOTE: For prescription of therapeutic footwear due to disease pathology resulting in neuropathy or peripheral artery disease. 55. CHECK APPROPRIATE DIABETIC/AMPUTATION RISK SCORE: ☐ RISK SCORE 2: PATIENT DEMONSTRATED SENSORY LOSS (<i>inability to perceive the Semmes-Weinstein 5.07 monofilament</i>), DIMINISHED CIRCULATION AS EVIDENCED BY ABSENT OR WEAKLY PALPABLE PULSES, FOOT DEFORMITY, OR MINOR FOOT INFECTION, & A DIAGNOSIS OF DIABETES. ☐ RISK SCORE 3: PATIENT DEMONSTRATED PERIPHERAL NEUROPATHY WITH SENSORY LOSS (<i>i.e., inability to perceive the Semmes-Weinstein 5.07 monofilament</i>), AND DIMINISHED CIRCULATION, AND FOOT DEFORMITY, OR MINOR FOOT INFECTION & A DIAGNOSIS OF DIABETES, OR ANY OF THE FOLLOWING BY ITSELF: (1) PRIOR ULCER, OSTEOMYELITIS OR HISTORY OF PRIOR AMPUTATION; (2) SEVERE PERIPHERAL VASCULAR DISEASE (PVD) (<i>intermittent claudication, dependent rubor with pallor on elevation, or critical limb ischemia manifested by rest pain, ulceration or gangrene</i>); (3) CHARCOT'S JOINT DISEASE WITH FOOT DEFORMITY; & (4) END STAGE RENAL DISEASE. NOTE: Only patients who are experiencing medical conditions noted in the risk scores can be prescribed therapeutic/diabetic footwear.
*ATTESTATION: I do hereby attest that the forgoing information is true, accurate, & complete to the best of my knowledge & I understand that any falsification, omission, or concealment of material fact may subject me to administrative, civil, or criminal liability. I do hereby acknowledge that VA reserves the right to perform the requested service(s) if the following criteria are met: (1) The patient agrees to receive services from VA (2) Service(s) are available at VA facility & are able to be provided by the clinically indicated date (3) It is determined to be within the patients best interest. Upon completion of the requested service(s), VA will provide all resulting medical documentation to the ordering provider. If all criteria listed are not true & VA agrees the service(s) are clinically indicated, VA will provide a referral for services to be performed in the community. I do hereby attest that upon receipt of order/consult results, I will assume responsibility for reviewing said results, addressing significant findings, & providing continued care.	
56. REQUESTING PROVIDER SIGNATURE (Required):	57. TODAY'S DATE (MM/DD/YYYY):

To facilitate timely review of this request, the most recent office notes & plan of care must accompany this signed form.

For more information please visit: https://www.va.gov/COMMUNITYCARE/providers/Care_Coordination.asp.

VA Community Care Medical Policies describe standard VA health care benefit for services and procedures that community providers may recommend as necessary for a Veteran. Prior to providing care, providers should use the Community Care Medical Policies as a reference when determining if a Veteran meets VA clinical criteria. When additional services are requested, Community Care Medical Policies will be used to determine approval by a clinical reviewer. Community Care Medical Policies & supporting information can be found at: <https://www.va.gov/COMMUNITYCARE/providers/Medical-Policy.asp>